

Ascension

Business Development Dossier

Industry	Healthcare Systems
Revenue	\$28B
Employees	150,000
Ideal Customer Profile Score	80

ICP SCORING MODEL



ICP = Signals + Revenue + Employees + Fit (max 100)

Signals (0-35)	Top 4 signals scored; each = base(60%) + recency bonus <30d: 40% <90d: 30% <180d: 20% older: 10%
Revenue (0-25)	\$500M-\$10B sweet spot -> 70-100% of 25 pts Below \$500M -> pro-rata x 0.5 Above \$10B -> 60%
Employees (0-20)	2,000+ preferred (70-100%) 500-2K partial (50-70%) Below 500 -> pro-rata x 0.3
Fit (0-20)	High-fit types: reorg, transformation, M&A, hiring, funding, partnership -> 5 pts ea (max 12) + diversity 2/type (max 8)

Prepared March 19, 2026

McChrystal Group

Leadership Advisory & Organizational Effectiveness

SECTION 1

Organization Snapshot

Legal Name Ascension Health

Headquarters St. Louis, Missouri

Founded 1999

Ownership

Nonprofit Catholic health system - sponsored by multiple Catholic religious institutes

Footprint

19 states plus the District of Columbia. Major markets: Texas, Florida, Michigan, Indiana, Wisconsin, Tennessee, Alabama. Divesting markets where scale is insufficient. AMSURG acquisition expands ambulatory presence to 34 states.

SECTION 1b

Company Overview

Ascension is the largest Catholic and one of the largest nonprofit health systems in the United States, operating ~90 hospitals and 2,600 care sites across 19 states. It is executing a historic pivot: divesting underperforming hospitals while acquiring AMSURG's 250+ ambulatory surgery centers to shift from inpatient to outpatient care delivery.

SECTION 2

Financial Health & Growth Stage

Understanding Ascension's financial position and trajectory is critical to framing any engagement conversation.

Revenue ~\$28B. Posted \$608M net income in first half of fiscal year (doubled year-over-year), signaling financial recovery after years of operating losses. Funded the \$3.9B AMSURG acquisition through a combination of debt and operating cash flow. Simultaneously divesting underperforming hospitals - sold 9 Illinois hospitals to Prime Healthcare, divested Michigan hospitals, reducing from 139 to ~90 hospitals. Strategy: shed inpatient overhead, invest in higher-margin outpatient care. Key risk: the divestiture-and-acquisition strategy must be executed simultaneously without organizational whiplash.

SECTION 3

Leadership Team Profiles

The following contacts are recommended as primary outreach targets - champion-level leaders who feel the organizational pain daily and can sponsor McChrystal internally.

Eduardo Conrado - President [PRIORITY TARGET]

Former CTO at Motorola Solutions; technology/transformation executive brought in from outside healthcare

Outreach rationale: As President with a technology/transformation background from outside healthcare, Conrado is the

operational change agent - he feels the integration pain daily and has the authority to champion McChrystal internally.

Unknown - SVP of Ambulatory Services / Integration [PRIORITY TARGET]

Unavailable

Outreach rationale: This person owns the AMSURG integration across 34 states - the most complex organizational challenge in the system.

Additional leadership team members relevant to the engagement:

Joseph Impicciche - CEO

Attorney; former General Counsel at Ascension before becoming CEO

SECTION 4

Organizational Culture & Structure

Culture and structure are often where McChrystal Group finds the deepest leverage. The following signals indicate how Ascension operates today and where friction exists.

Organizational Structure

Large Catholic health system with traditional hierarchical hospital administration structure. The AMSURG acquisition introduces an entirely different operating model: ASCs are lean, physician-led, procedure-focused operations - culturally and operationally distinct from hospital-based care. Ascension's mission-driven, Catholic identity adds a values dimension that must be preserved through the transformation.

Culture Signals

Ascension's Catholic mission and values orientation creates a strong institutional identity but can slow decision-making through consensus and stakeholder alignment requirements. Hospital-based culture is hierarchical, risk-averse, and clinician-led. ASC culture (AMSURG) is entrepreneurial, physician-owned, and volume-driven. The cultural gap between these models is significant. Glassdoor reviews for Ascension show mixed sentiment: appreciation for mission-driven work but frustration with bureaucracy, pace of change, and the emotional toll of hospital divestitures on long-tenured employees. Employee morale in divested markets is a concern. [INFERRED: physician resistance to centralized control over ASC operations is likely]

SECTION 5

Recent News & Trigger Events

The following developments from the past 18 months create urgency and provide natural conversation entry points for engaging Ascension.

1. 2025-06-01 - Ascension announces \$3.9B AMSURG acquisition - 250+ ASCs across 34 states

Largest ambulatory acquisition in Catholic healthcare history; transforms the care delivery model

2. 2026-02-01 - Ascension reports \$608M net income (doubled YoY) - financial recovery fuels expansion

Financial health enables the investment but raises expectations for ROI on the AMSURG deal

3. 2024-07-01 - Ascension sells 9 Illinois hospitals to Prime Healthcare

Signals commitment to portfolio rationalization - divesting where scale is insufficient

4. 2025-01-01 - Ascension divests Michigan hospitals, reducing total from 139 to ~90

Continued divestiture demonstrates the 'shrink to grow' strategy is structural, not opportunistic

5. 2024-05-01 - Ascension suffers major cyberattack disrupting operations across multiple hospitals

Cyber vulnerability exposed operational fragmentation - reinforces need for integrated systems and shared consciousness

SECTION 6**McChrystal Fit Assessment**

massive active trigger (\$3.9B AMSURG deal), clear organizational challenge (hospital + ASC integration), proven budget capacity (\$28B system), and the hospital-ASC integration mirrors McChrystal's core capability - connecting autonomous units into shared consciousness.

Primary Problem

Executing a simultaneous two-directional transformation: integrating 250+ independently-operated ambulatory surgery centers into a Catholic health system while divesting ~50 hospitals - requiring two fundamentally different organizational capabilities (acquisition integration and managed divestiture) running in parallel.

Best McChrystal Capability Fit

Team of Teams operating model - connecting 250+ autonomous ASC operations (lean, physician-led, entrepreneurial) with Ascension's hospital-based infrastructure (hierarchical, mission-driven, consensus-oriented) while preserving the autonomy that makes ASCs successful. This is the exact challenge McChrystal solved: decentralized units maintaining operational independence within shared strategic consciousness.

Likely Objections to Engagement

Catholic health system governance may be slow to approve external advisory engagements
Multiple simultaneous initiatives (AMSURG, divestitures, cyber recovery) may fracture attention and budget
Healthcare advisory market is crowded with industry-specific firms (Kaufman Hall, Chartis, Huron)

Competitive Advisory Landscape

Kaufman Hall or Chartis Group (healthcare-specific strategy advisory)
McKinsey Healthcare practice (large health system transformation)
Huron Consulting (healthcare operations and integration)
Deloitte/PwC healthcare practices

SECTION 7

Conversation Entry Points

These questions are designed for a Senior Partner's first conversation. Each opens a thread that naturally leads to McChrystal Group's capabilities without a direct pitch.

1. How are you thinking about preserving ASC operational autonomy - the speed, physician-ownership feel, volume-driven culture - while connecting them to Ascension's infrastructure and mission?

This question gets at the core tension: centralize too much and you kill what makes ASCs profitable; decentralize too much and you don't get integration value.

2. With divestitures and acquisitions running simultaneously, how are you managing organizational attention - are the same leaders responsible for both, and is that sustainable?

Acknowledges the two-directional complexity that's unique to Ascension's situation.

Recommended Meeting Framing

Position as a conversation about integrating autonomous operating units - McChrystal has done this in military contexts where autonomous special operations teams had to coordinate with conventional forces. The ASC-hospital integration is the healthcare equivalent.

SECTION 8

Brand Insights & Market Positioning

Brand strategy reveals organizational priorities and coordination challenges. The following analysis connects Ascension's market positioning to potential McChrystal engagement opportunities.

Ascension's brand is rooted in Catholic healthcare mission - compassionate care, community service, and ethical principles. The shift to ambulatory care challenges this brand identity: ASCs are associated with efficiency and volume, not community hospital care. The brand must evolve from 'your neighborhood Catholic hospital' to 'your comprehensive Catholic healthcare network.' Major brand risk: divestitures in communities where Ascension was the primary healthcare provider create reputational damage and mission-alignment questions. [INFERRED: brand investment in communicating the ambulatory pivot to communities, physicians, and employees is significant and requires organizational coordination across 34 states]

SECTION 9

Deep McChrystal Group Fit Analysis

This is the strategic case for pursuing Ascension. It synthesizes findings from all previous sections into a comprehensive engagement thesis with specific fit dimensions, expected outcomes, stakeholder mapping, and a phased pursuit strategy.

9a. Fit Dimensions

Fit Dimension 1: Hospital-ASC Integration

250+ independently-operated ASCs must connect to a Catholic hospital system without losing the operational agility that makes them profitable. This is McChrystal's defining challenge: connecting autonomous units into shared consciousness while preserving decentralized execution. Healthcare-specific advisors (Kaufman Hall, Chartis) understand the industry but lack the operating model methodology McChrystal brings.

Fit Dimension 2: Two-Directional Transformation

Divesting ~50 hospitals while acquiring 250+ ASCs requires two distinct organizational capabilities running simultaneously. The emotional toll (closing legacy hospitals while opening new ASCs) creates organizational whiplash. McKinsey could model the portfolio strategy; McChrystal builds the organizational resilience to execute both directions at once.

Fit Dimension 3: Mission-Driven Culture Under Business Transformation

Ascension's Catholic identity is both a strength (purpose-driven workforce) and a constraint (stakeholder governance, consensus culture, ethical guidelines that may slow commercial decisions). McChrystal's experience in mission-driven organizations (military) maps directly to this challenge.

9b. Cumulative Case

Signal chain: hospital divestitures (2024-2025) -> \$3.9B AMSURG acquisition -> financial recovery (\$608M net income) -> ambulatory-first transformation -> cyber vulnerability exposure -> 34-state integration challenge. Each signal amplifies the organizational complexity.

Revenue potential: \$1M initial diagnostic + Team of Teams workshops for top 50 leaders (4-6 months), \$2M-\$3M expanded engagement across regional markets and ASC integration (12-18 months), \$750K+/year ongoing advisory.

9c. Enterprise Issues

1. ASC-Hospital Cultural Collision: ASC operators are lean, physician-led, volume-driven entrepreneurs. Hospital administrators are hierarchical, mission-driven, consensus-oriented. Merging these cultures without homogenizing either is the central challenge.
2. 34-State Integration Complexity: AMSURG operates across 34 states; Ascension across 19. The geographic expansion adds regulatory, market, and cultural complexity.
3. Divestiture-Induced Organizational Grief: Closing legacy hospitals - many serving communities for decades - creates emotional toll on employees, physicians, and communities that reverberates throughout the system.
4. Physician Autonomy Preservation: ASC physicians chose the ASC model specifically for its autonomy. Any perception that Ascension will bureaucratize their operations risks physician defection.
5. Cyber Vulnerability: The 2024 cyberattack exposed operational fragmentation. Adding 250+ ASCs to the technology infrastructure without a unified cybersecurity framework compounds this risk.
6. Catholic Mission Alignment: Integrating ASCs that perform procedures potentially in tension with Catholic ethical directives requires careful governance navigation.

9d. Expected Outcomes

1. ASC Integration Operating Model - Shared consciousness framework connecting 250+ ASCs to Ascension infrastructure while preserving ASC operational autonomy. Measurable: unified quality and safety standards across all ASCs within 12 months while maintaining ASC-level margin performance.
2. Two-Directional Transformation Framework - Organizational architecture for simultaneous divestiture and acquisition execution. Measurable: complete AMSURG integration milestones on schedule while executing remaining hospital divestitures without disruption.
3. Cultural Bridge Program - Facilitate understanding between hospital and ASC cultures. Measurable: physician retention in AMSURG ASCs above 95% through first 18 months.
4. Regional Integration Leadership - Develop shared consciousness across 34-state footprint. Measurable: cross-site collaboration metrics and knowledge sharing increase 30% within 12 months.
5. Mission-Aligned Transformation Narrative - Help Ascension articulate how the ambulatory pivot advances (not abandons) the Catholic healthcare mission. Measurable: employee mission alignment scores maintained through transformation.

9e. Key Stakeholders & Business Unit Map

Hospital Operations

Leader: Unknown

Function: Acute care across ~90 hospitals

McChrystal Relevance: Legacy operations - divestiture management and cultural stability

Ambulatory Services / AMSURG

Leader: Unknown

Function: 250+ ASCs across 34 states

McChrystal Relevance: Integration epicenter - priority engagement zone

Office of the President

Leader: Eduardo Conrado

Function: Strategic transformation

McChrystal Relevance: Executive sponsor - transformation architect

Mission Integration

Leader: Unknown

Function: Catholic identity and ethics

McChrystal Relevance: Ensures transformation aligns with Catholic mission

IT / Cybersecurity

Leader: Unknown

Function: Technology infrastructure

McChrystal Relevance: ASC technology integration and cyber resilience

Engagement priorities:

1. Eduardo Conrado, President (transformation champion with outside-healthcare perspective)
2. SVP Ambulatory Integration (daily integration execution pain)

3. Regional market presidents (3-4 key markets where ASC-hospital overlap is highest)

9f. Opportunity Thesis

Why Now: AMSURG deal closing 2025-2026. Integration is happening now. The first 12-18 months determine whether the ASCs maintain their operational excellence or get bureaucratized into hospital-system mediocrity.

Why McChrystal, Not McKinsey: Healthcare-specific advisors (Kaufman Hall, Chartis) understand the industry but optimize within the hospital paradigm. McKinsey would design the portfolio strategy. McChrystal solves the operating model challenge: how do 250 autonomous units maintain their entrepreneurial edge while sharing consciousness with a hierarchical hospital system? This is the literal definition of Team of Teams.

Phased Engagement:

- Phase 1 (\$750K-\$1M, 4-6 months): Leadership alignment + Team of Teams workshops for top 50 leaders. Beachhead: 3-4 markets where Ascension hospitals and AMSURG ASCs overlap - this is where the cultural collision is visible and wins are most meaningful.
- Phase 2 (\$1.5M-\$2.5M, 12-18 months): ASC integration operating model across all 34 states.
- Phase 3 (\$500K-\$750K/year): Ongoing advisory through continued transformation.

Multi-Threaded Pursuit:

- Thread 1: Eduardo Conrado, President - direct outreach on transformation operating models; his Motorola background means he'll appreciate a non-healthcare perspective
- Thread 2: SVP of Ambulatory/Integration - hands-on integration support angle
- Thread 3: McChrystal network - check for Catholic healthcare board connections, military/veteran physicians in Ascension system